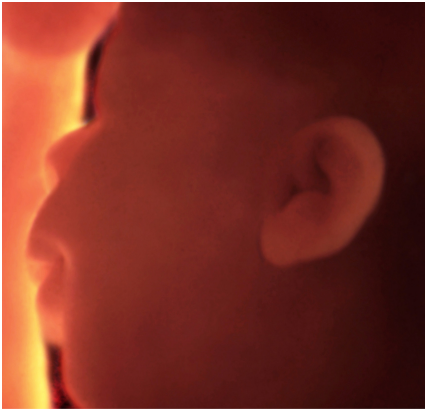




YOU ARE **36 WEEKS AND 4 DAYS**

24 days to go

YOUR BABY TODAY

Your baby is now more well equipped for the time after birth. Their ears are fully formed on the outside and inside and they are used to hearing the sound of your blood circulating and your heart beating, and they recognize your voice.

“Your baby is losing the downy hair that they’ve had for several weeks, but if they’re born now there may still be some visible.”

Lanugo is extremely fine hair that covers your baby’s body and, unlike adult hairs on the body, it is not associated with sweat glands. Lanugo hairs begin to be shed now since they are lost into the amniotic fluid in the last few weeks before birth. Your baby then swallows these lanugo hairs in the amniotic fluid, but this is not a cause for concern: these hairs provide your baby with an important source of protein, essential to his development. It’s been estimated that two-thirds of the protein in the fluid is swallowed and absorbed by the baby’s gut each day, providing 15 percent of his protein needs.

The fine lanugo hairs are gradually replaced by vellus hairs, which are short, soft, nonpigmented hairs (often seen more on women and children). Terminal hair is the thicker, coarser, and longer hair that first grows on your baby’s eyebrows, then his eyelashes, then his scalp. In adults, facial hair (beards), armpit, and pubic hair is terminal hair.

NEEDING THE BATHROOM

In the third trimester you’ll be frequenting the bathroom regularly, as you did in the first trimester. At this stage, it’s due to the increasingly heavy baby pushing down on your bladder from above. If you find that it hurts when you expel urine, you may have a urinary tract infection and should contact your doctor for a test.

ASK A DOCTOR

I’ve heard about doctors “turning” breech babies. How does this work? Some obstetricians may try to turn a baby in late pregnancy using external cephalic version (ECV), which has a success rate of around 65 percent. They’ll try gently to guide the baby into a head-down position by pressing their hands on the mother’s abdomen, using ultrasound as a guide. You may be given medication to relax the uterine muscles. You will be scanned first and if the baby is in an